

Booking/first antenatal visit continued:

Assess	Note
Haemoglobin (Hb)	Give iron according to Hb ↗ 163. Refer if: - If Hb < 6, or Hb 6-7.9 <i>with symptoms</i> (dizzy, pulse > 100, difficulty breathing at rest): refer same day to regional hospital. - If Hb 6-7.9 without symptoms: refer within 1 week to next level of care clinic. - If booking late and Hb < 10 at ≥ 36 weeks: refer to next level of care clinic and arrange delivery at hospital.
Rapid rhesus (Rh)	If rhesus negative, send Coombs test to check for antibodies: if Coombs positive, refer. If Coombs negative, patient will need anti-D immunoglobulin IM after delivery.
Syphilis	- Ideally, use rapid fingerprick test, as result immediately available. If HIV negative or unknown, use a dual HIV syphilis rapid test, if available. If syphilis positive ↗ 53. If HIV positive ↗ 110. - If rapid tests unavailable: send blood for syphilis serology (RPR). On request form, write: "If RPR titre 1:4 or less, do specific syphilis test on same specimen." If syphilis positive ↗ 53.
HIV	- If HIV negative or status unknown, test for HIV ↗ 110. If test negative, consider need for PrEP ↗ 106. - If HIV positive, start ART same day if not on ART yet ↗ 111. If on ART, switch to DTG-based regimen if not on already ↗ 117. Offer couple/partner testing.
Viral load (VL) if HIV positive	- If on ART for ≥ 3 months: do VL at this visit, regardless of previous tests. Follow up result at next visit (ideally within 1 week) ↗ 163. - If on ART for < 3 months: do VL at 3 months on ART or at delivery if this is sooner.
Hepatitis B (HBsAg)	Manage according to result ↗ 120.
Cervical screen	- If ≥ 20 weeks: delay cervical smear. Plan to do it at 6-week postnatal visit. - If < 20 weeks: - If HIV negative: if patient ≥ 30 years <i>and</i> no screen in past 10 years, do cervical screen ↗ 55. - If HIV positive: do cervical screen at HIV diagnosis (regardless of age), then 3-yearly ↗ 55.

Note: fill in the code 'C#PMTCT' on the blood request form for viral load to be identified as that of pregnant woman

Continue to advise and treat the pregnant patient →163.

Follow-up antenatal visits

Assess the pregnant patient at booking/first visit ↗ 160 and 7 follow-up visits around 20, 26, 30, 34, 36, 38, 40 weeks. Review at 41 weeks if undelivered.

Assess	When to assess	Note
Symptoms	Every visit	Manage symptoms as per symptom page. Check if patient needs urgent attention ↗ 159.
Gestation ¹	Every visit	If ≥ 40 weeks, advanced midwife/doctor to review: if sure of dates, to go to hospital at exactly 41 weeks for induction (give referral letter). If unsure of dates, refer.
Fetal movements	Every visit from 20 weeks	- If reduced or absent fetal movements, listen for fetal heartbeat: if fetal heart beat not heard, refer. - If fetal heart beat heard, arrange for cardiotocograph (CTG). Refer if not available at facility. Ideally, advanced midwife to perform and interpret CTG:
TB	Every visit	- Check for TB symptoms at every visit: if cough, weight loss/poor weight gain or fever, exclude TB ↗ 92. If patient has TB, refer to next level of antenatal care clinic. - If HIV positive, check TB NAAT result sent at first visit (if not done, do at this visit, even if no symptoms): - If TB NAAT positive, start TB treatment and refer to next level of care antenatal clinic. - If TB NAAT negative (or unable to produce sputum) and: - TB symptoms: if CD4 ≤ 200 or WHO stage 3 or 4 disease, do a urine LAM². If LAM positive, start TB treatment and refer. If CD4 > 200 or LAM negative, refer/discuss. - No TB symptoms: start ART, if not already done ↗ 113 and TB preventive treatment (TPT) ↗ 113.
Mental health	Every visit	- In the past 2 weeks, has patient: 1) been unable to stop worrying or thinking too much 2) felt down, depressed, hopeless? If yes to any ↗ 143. - In the past 2 weeks, has patient had thoughts or plans to harm herself? If yes ↗ 83.
Alcohol/drug use	Every visit	- Any alcohol/drug use is risky for the baby. - In past year, has patient: 1) drunk ≥ 4 drinks³/session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any, discuss/refer.

Continue to assess the pregnant patient →162.

¹Use obstetric wheel to determine gestation, based on estimated date of delivery (EDD). ²Urine LAM (lipoarabinomannan): urine test used to detect active TB in patients with low CD4s. ³One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ⁴Symphysis-fundal height. ⁵Glucose in urine, BMI ≥ 32, age ≥ 40 years, previous diabetes in pregnancy, family history of diabetes, previous unexplained stillbirth, previous baby ≥ 4000g, polyhydramnios, SFH large for gestational age, Indian ethnicity.